

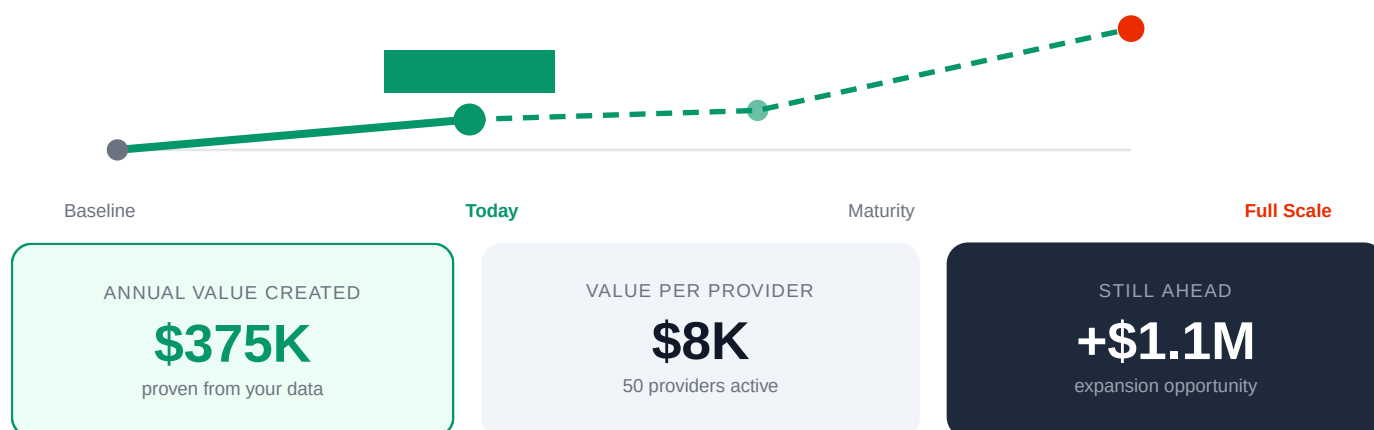
ABC Hospital

Your Value Realization Report
4 months of partnership. Real results.

THE STORY SO FAR

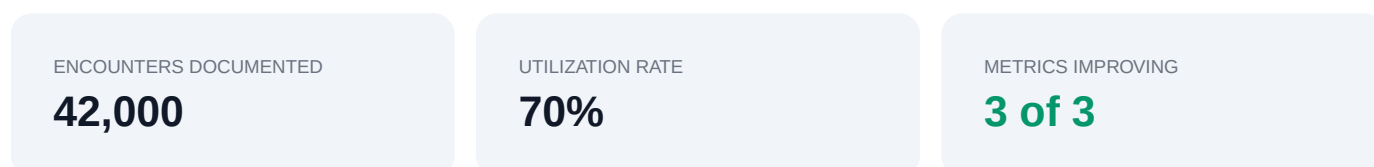
Your Abridge Journey

Half a year in, and the results speak for themselves. Your providers are spending less time on documentation and more time on what matters.



BY THE NUMBERS

What Your Data Shows



"Every number in this report comes from your actual experience—measured changes in how your providers work, how they feel, and the value they create. This isn't a projection. It's your reality."

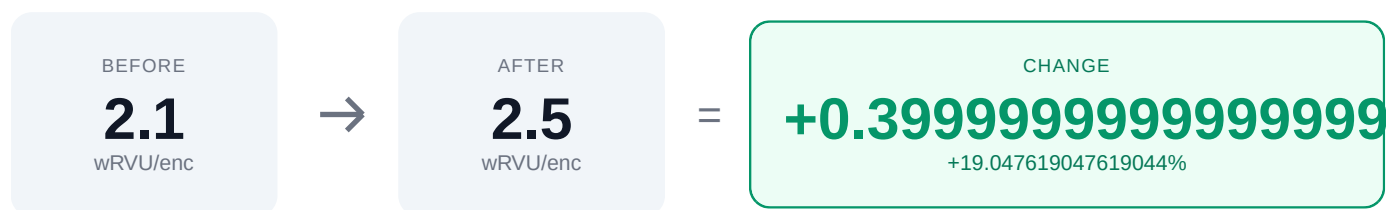
wRVU Per Encounter

Capturing the value that was always there.

REVENUE CAPTURE

The Transformation

Your data tells the story: 42,000 encounters documented by 50 providers.



THE REAL STORY

Why This Matters

This is the kind of improvement that gets CFO attention. Documentation is now capturing the true complexity of care—revenue that was always earned but never captured. Your providers aren't working harder; they're just getting credit for what they already do.

Here's what we're seeing: A 19.0% lift in wRVU typically means your documentation is now capturing clinical complexity that was always there—providers aren't doing more, the notes are just reflecting reality better. Your 19% lift is above our typical 3-7% range, which is interesting. This could mean several things: your baseline documentation may have been particularly sparse, or you have complex patient populations that benefit more from thorough capture. It's worth exploring—if you can validate the baseline methodology, this becomes a powerful proof point.

ANNUAL VALUE CREATED

$$+0.40 \text{ wRVU/enc} \times 42,000 \text{ encounters} \times \$33/\text{wRVU} \times 50\% \text{ attribution}$$

\$277K/year

We use Medicare's \$33 conversion factor and 50% attribution to Abridge. If your payer mix is commercial-heavy (where conversion factors run \$45-65), actual revenue impact may be 30-50% higher.

INDUSTRY BENCHMARK

Typical range: 3-7% lift. Your result is above typical range.

NOTE

Your results exceed our typical range. This isn't necessarily wrong—some organizations do see higher lift, especially if baseline documentation was sparse. But we recommend confirming: Was baseline measured the same way as current? Were there other coding or documentation initiatives during this period? Is the provider population consistent between periods?

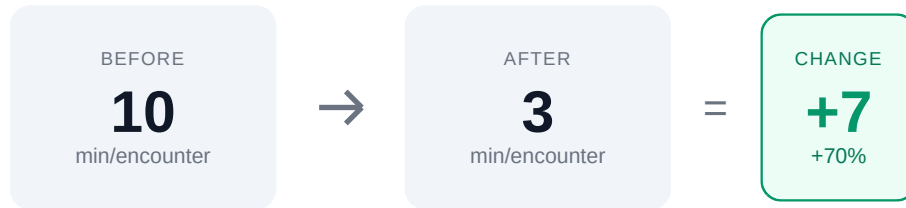
Time in Notes

Every minute saved is a minute earned.

TIME EFFICIENCY

The Transformation

Your data tells the story: 42,000 encounters documented by 50 providers.



THE REAL STORY

Why This Matters

7 minutes per encounter doesn't sound like much—until you multiply it by thousands of visits. That's hours back every week. Hours that used to be spent typing, now spent with patients, with family, or just breathing.

Let's think about what 7 minutes saved really means: That's 4,900 hours annually—time that was previously locked up in documentation. The interesting question is: where does that time actually go? We've seen organizations channel it three ways: back to patients (more access, longer visits), back to providers (better work-life balance), or to operations (overtime reduction). Which path makes sense for you depends on your current priorities. The 7-minute reduction you're seeing is meaningful—it's the kind of lift that providers notice in their daily rhythm.

ANNUAL VALUE CREATED

$7 \text{ min saved} \times 42,000 \text{ enc} \div 60 \times \$150/\text{hr} \times 10\%$

\$98K/year

INDUSTRY BENCHMARK

Typical range: 3-5 min reduction. Your result is above typical.

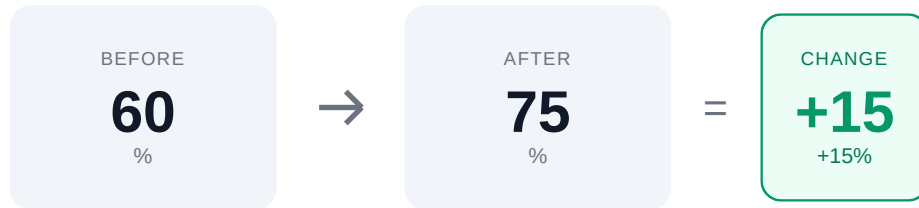
Same-Day Chart Closure

Charts that close when you do.

TIME EFFICIENCY

The Transformation

Your data tells the story: 42,000 encounters documented by 50 providers.



THE REAL STORY

Why This Matters

More charts closing same-day means less cognitive burden carried overnight. It means leaving work at work. The ripple effects of this change extend far beyond the EHR.

Chart closure is one of those metrics that tells a bigger story. When charts close same-day, it usually means providers are documenting in real-time rather than batching at the end of the day (or week). Your 15 percentage point improvement suggests the workflow is clicking—providers are trusting the AI draft enough to finalize notes between patients. Beyond the workflow signal, there's a downstream revenue impact: faster closure means faster billing cycles and reduced compliance exposure. Worth watching as your deployment matures.

INDUSTRY BENCHMARK

Typical range: 10-20 percentage point improvement. Your result is within typical range.

What Could Be

You've proven value. Now let's talk about scale.

THE OPPORTUNITY

From Pilot to Platform

You've built a strong foundation with 50 providers. Extending to 150 means 100 more clinicians spending less time in the EHR and more time with patients. That's the kind of change that defines culture.

Provider Coverage

50 of 150 (33%)



● Active (\$375K)

○ Opportunity (+\$1.1M)

TODAY'S VALUE

\$375K

50 providers

FULL SCALE VALUE

\$1.5M

150 providers

THE UNLOCK

+\$1.1M

additional annual value

THE ECONOMICS

Value Scales With You

VALUE PER PROVIDER

\$8K

TARGET UTILIZATION

81%

SCALE FACTOR

3.0×

The Question Isn't If—It's When

Every month you wait is another month of documentation burden for 100 providers who could be experiencing the same relief your current users already have.

At \$10K per provider per year, the math works. But the real win? A better day-to-day for your clinical team.

The Bottom Line

A snapshot of ABC Hospital's Abridge journey—ready to share.

EXECUTIVE SUMMARY

What We've Built Together

4 months. 50 providers. 42,000 encounters. Real results.

Value Created

ANNUAL VALUE

\$375K

proven from your data

PER PROVIDER

\$8K

annual value each

EXPANSION POTENTIAL

+\$1.1M

at full scale

Your Deployment

ACTIVE PROVIDERS

50

using Abridge

UTILIZATION

70%

of eligible encounters

PARTNERSHIP

4

months and counting

PERFORMANCE INSIGHTS

What's Working

- Revenue capture is responding—19% wRVU lift suggests documentation is better reflecting clinical complexity
- Providers are reclaiming time—7 min/encounter freed up, which compounds across your volume
- Real-time documentation emerging—15pt closure improvement suggests workflow is clicking

Opportunities to Optimize

- Utilization at 70%—worth exploring what's keeping remaining providers from adopting. Training gaps? Specialty-specific barriers?
- wRVU lift above typical—exciting if validated. Consider auditing baseline methodology to confirm this is real lift vs. measurement change

You've proven the value. You have the data. The question now is simple: how fast can you bring this to the rest of your organization?

Ready to take the next step?

Contact your Abridge partner to discuss expansion